February, 19 2026

Dockets Management Staff
Department of Health and Human Services
Office of the Secretary
330 C Street S.W.
Washington, D.C. 20201

Re: Docket No. 2025-23641 (90 FR 60108)
RIN 0955-AA13
Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Sir or Madam,

Veeva Systems appreciates the opportunity to provide comments to the Department of Health and Human Services (HHS) in response to the Request for Information (RFI) on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care.

Veeva Systems is a global provider of cloud-based software for the life sciences industry, supporting more than 1,000 pharmaceutical, biotechnology, and medical device organizations worldwide. Our platforms support clinical research, regulatory submissions, quality management, and real-world evidence generation. Veeva works extensively with organizations that are building, evaluating, and deploying AI-enabled capabilities across regulated and clinical workflows. Accordingly, our comments focus on AI used in clinical care and reflect our experience with data standards, interoperability, governance, and operational implementation.

Veeva supports HHS's "OneHHS" approach to artificial intelligence and commends the Department's efforts to establish a forward-leaning, predictable, and proportionate framework that accelerates responsible AI adoption while preserving patient privacy, civil rights, and public trust. Further, we agree with the Administration's thoughts in America's AI Action Plan that healthcare has been "especially slow to adopt [AI] due to a variety of factors, including distrust or lack of understanding of the technology, a complex regulatory landscape, and a lack of clear governance and risk mitigation standards." While we anticipate rapid acceleration due to AI in areas such as drug discovery and care delivery, we believe that clinical trials and clinical research will continue to be a bottleneck in the process of getting treatments to patients faster. Accelerating and innovating in how clinical trials are performed, with a focus on the technology used by research sites, will allow the promise of AI across healthcare to be more fully realized. Our responses below, to a subset of the specific questions, are organized to reflect HHS's stated focus on regulation, reimbursement, and research and development.

Question 1: Biggest barriers to private sector innovation and adoption

The most significant barriers to innovation and adoption of AI in clinical care include challenges related to workflow integration, governance and accountability, trust, and alignment with existing clinical operations. Many AI tools struggle to fit cleanly into care delivery environments, creating uncertainty for clinicians and administrators responsible for patient safety, compliance, and oversight.

In addition to these well-recognized barriers, limitations in the clinical research and evidence-generation ecosystem constrain how AI systems are developed, validated, and monitored before and after deployment in care settings. While clinical research and clinical care serve distinct functions, much of the evidence that informs clinical decision-making is generated within routine care environments, including community-based sites, sponsor-led studies, hybrid care-research settings, and real-world clinical workflows.

Clinical data used to train and evaluate AI models often lacks consistent structure, provenance, and longitudinal linkage across research and care environments. Although interoperability in routine clinical care has improved, clinical research data remains fragmented across sponsors, sites, and technology platforms. This fragmentation limits the availability of representative, high-quality datasets needed to support the safe, scalable, and trustworthy use of AI in clinical care.

Together, these factors create friction not only in deploying AI tools in care delivery, but also in establishing the evidence base and ongoing oversight required to sustain trust, accountability, and long-term adoption.

Question 2: Regulatory, payment, and programmatic changes to prioritize

HHS should prioritize regulatory and programmatic actions that strengthen the data and evidence foundations upon which safe and effective AI use in clinical care depends, rather than focusing primarily on prescriptive requirements for AI models themselves.

From a regulatory and programmatic perspective, HHS should continue aligning AI-related policy with existing interoperability frameworks such as the United States Core Data for Interoperability (USCDI) and HL7® FHIR®, while ensuring these frameworks adequately support research-specific requirements, including data provenance, auditability, and linkage across care and research settings. Clear and consistently applied expectations—when paired with common standards, implementation guidance, and conformance mechanisms—can reduce friction for organizations developing and evaluating AI systems intended for use in clinical care by limiting interpretive variation and redundant data transformation.

HHS should also consider whether greater consistency and transparency in clinical research technology would support downstream AI adoption in care delivery. In contrast to routine clinical care—where EHR certification has helped establish baseline interoperability and security expectations—clinical research technologies remain highly fragmented. Exploring voluntary, standards-based approaches to interoperability and data quality for modern clinical research systems could improve the reliability and representativeness of data used to train, validate, and monitor AI deployed in clinical settings.

While reimbursement policy influences the pace of adoption, particularly for provider-facing tools, the ability of AI to deliver sustained value in clinical care ultimately depends on the quality, traceability, and governance of the underlying data. Programmatic investments that strengthen research-to-care data flows would therefore have a durable impact across regulatory, clinical, and operational contexts.

Question 5: Supporting private-sector accreditation and certification

HHS can support effective and innovative use of AI in clinical care by establishing clear, federally led expectations for interoperability, security, and data quality in the clinical research technologies that generate evidence supporting the development, approval, and use of medicines in care delivery.

Routine clinical care has benefited from government-defined certification programs that created consistent baseline expectations across a fragmented technology market. By contrast, clinical research technologies—despite producing data that informs regulatory decisions, clinical guidelines, and increasingly AI-enabled decision support—operate without a comparable, widely adopted federal framework.

HHS should consider whether a government-developed certification or conformance program for modern clinical research systems could strengthen the structure, provenance, and auditability of research-derived data used to develop, validate, and monitor AI deployed in clinical care. Such an approach would not regulate AI models themselves, but would improve consistency across the upstream data infrastructure on which clinical AI depends. Over time, this could reduce redundant data transformation, support more representative datasets, and increase trust in AI-enabled clinical decision support.

Question 6: Where AI has met or fallen short of expectations

From Veeva's perspective, AI has met expectations where use cases are narrowly defined, operationally integrated, and supported by reliable underlying data. These include applications that streamline administrative processes, support clinical operations, or analyze population-level trends without directly substituting for clinical judgment.

Conversely, AI has fallen short where underlying data is incomplete, poorly governed, or insufficiently representative, limiting the ability to validate performance and sustain trust over time. In many cases, these shortcomings reflect upstream gaps in the generation, standardization, and reuse of clinical research and real-world evidence rather than limitations of AI techniques themselves.

AI tools with the greatest potential to improve outcomes and reduce burden are those that are grounded in high-quality evidence, transparently governed, and designed to complement existing clinical workflows—particularly where they support the safe and effective use of medicines in real-world care settings.

Question 8: Role of interoperability in accelerating AI development

Enhanced interoperability would accelerate the responsible development and use of AI in clinical care by strengthening the connection between clinical care delivery and the clinical research activities that generate evidence supporting the use of medicines in practice. While existing interoperability frameworks have meaningfully improved data exchange for treatment purposes, opportunities remain to better support innovation, research, and evidence generation that increasingly occur within routine care environments.

Frameworks such as TEFCA, together with standards like HL7® FHIR® and USCDI, could play a larger role in reducing fragmentation between clinical care and clinical research. HHS should consider whether expanding TEFCA to more explicitly support research exchange purposes would enable secure, standardized access to clinical data for research activities embedded in care delivery, including cohort identification, trial feasibility, and evidence generation.

By enabling research exchange at scale, TEFCA could support AI-enabled identification of more diverse and representative patient populations across community-based and rural care settings, helping address long-standing challenges in trial participation and data representativeness. Over time, this would improve the quality of data used to develop, validate, and monitor AI systems that inform the use of medicines in clinical care.

In a future state, stronger interoperability between research and care systems could also enable insights generated through clinical research and real-world evidence to be more rapidly translated back into care delivery, supporting AI-enabled clinical decision support that reflects up-to-date evidence and real-world patient populations.

Conclusion

Veeva supports HHS's efforts to accelerate the responsible adoption of artificial intelligence in clinical care. As AI is increasingly used to inform clinical decision-making and the use of medicines, its safety, effectiveness, and trustworthiness will depend not only on model performance, but on the quality, interoperability, and governance of the clinical research and evidence-generation systems that underpin it.

HHS has a meaningful opportunity to advance AI adoption by strengthening the data foundations that connect clinical research and clinical care, including through clear, federally led expectations for interoperability, data quality, and consistency across modern clinical research technologies. Targeted action in these areas can improve the representativeness, reliability, and reuse of evidence used to develop, validate, and monitor AI systems in real-world care settings.

Veeva appreciates the opportunity to provide focused input on these issues and welcomes continued engagement with HHS as it considers policy approaches that support innovation while maintaining trust, accountability, and patient safety.

If you have any questions regarding these comments, please contact me at Anthony.Corso@veeva.com or (614) 401-2914.

Sincerely,

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